

SC Health and Human Services

Effective Date: November 2014

Division of Community and Facility Services

Revised: May 2017

Incontinence Supplies State Plan Program

Policy and Procedure

**I. Introduction:**

The purpose of the Incontinence Supplies State Plan Program is to provide incontinence supplies to those eligible persons who are incontinent of bowel and/or bladder. The Incontinence Supplies Program is a Medicaid State Plan service. This program identifies persons who are eligible for the service and authorizes these supplies.

An individual may have more than one program application and an Incontinence Supplies application open at the same time if his/her condition requires. It is important for workers to note all Phoenix applications prior to initiating and terminating case actions.

## **II. Application Process:**

### **Centralized Intake:**

A referral for Incontinence Supplies Program (IS) may originate from any knowledgeable source. Referrals may be made to Community Long Term Care Centralized Intake Team via telephone, written, walk-in or electronically at [https://phoenix.scdhhs.gov/cltc\\_referrals/new](https://phoenix.scdhhs.gov/cltc_referrals/new).

**State workers and provider case managers are required to make all incontinence supplies referrals electronically.**

New applications for Incontinence Supplies Program (IS) will be processed by the CLTC Centralized Intake Team utilizing the existing process for intake. If the applicant is only seeking incontinence supplies, the application will be assigned to an Incontinence Supplies Specialist.

In order to meet intake criteria, the applicant must have incontinence listed as a referral reason and be a current Medicaid recipient as indicated by the check box on the application screen. The applicant for Incontinence Supplies Program is required to have Medicaid as the Incontinence Supplies Program is a Medicaid state plan service.

NOTE: When the applicant has APPLIED for Medicaid Eligibility Category 15 MAO, but has not been approved per eligibility workflow notification, the referral for IS will be inappropriate and is to be terminated.

NOTE: When an enrolled waiver participant who will be category 15 (MAO) and is within the required 30 day waiting period, the referral source should make a comment in the IS referral comments section indicating the applicant is an approved Category 15 MAO participant who is in the 30 day wait period.

NOTE: Prior to enrollment, the intake worker will also review the applicant's Medicaid Eligibility RSP information. If the applicant is enrollment in managed care then the referral should be terminated and a letter sent for the participant to contact the managed care plan's member services regarding the need for incontinence supplies.

## **III. Medical Necessity Criteria for Incontinence Supplies State Plan Program:**

The following criteria must be met for an applicant to receive incontinence supplies:

1. Must be age 4 or older.
2. Inability to control bowel or bladder function. This must be confirmed in writing by a Physician only. (Nurse Practitioner, Registered Nurse or Physician Assistant signatures not acceptable.)
3. The Physician Certification of Incontinence form must be obtained from the physician indicating the service is medically necessary.
4. The Physician Certification of Incontinence is valid for 365 days or less if the Physician so indicates. In order for the service to continue, a new Physician Certification of Incontinence form must be

obtained every 365 days unless the Physician indicates a shorter time frame. The Incontinence Supplies provider is responsible for obtaining and tracking this form.

5. The participant's assessment will indicate the frequency of incontinence and this will determine the amount of supplies authorized.

#### IV. Frequency of Incontinence Supplies Definitions:

Bowel rating:

- If the individual has incontinent bowel episodes once a week, select Occasionally Incontinent.
- If the individual has incontinent bowel episodes two-to-three times a week, select Frequently Incontinent.
- If the individual has incontinent bowel episodes almost all of the time or has an ostomy, select Incontinent (total).

Bladder rating:

- If the individual has incontinent bladder episodes twice a week, but not daily, select Occasionally Incontinent.
- If the individual tends to have incontinent bladder episodes daily, but some control, select Frequently Incontinent. If the individual is on a toileting schedule, meaning the person is taken to the bathroom every two to three hours, the user may select Frequently Incontinent.
- If the individual has inadequate bladder control almost all of the time or has an indwelling catheter or ostomy, select Incontinent (total).

#### **Ostomy and Catheter Requirements:**

If the individual has an ostomy or catheter for urinary control **AND** has an ostomy for bowel control, neither diapers, nor briefs nor incontinence pads may be authorized. In this situation, under pads are the only product that should be authorized. Wipes are discussed in a later section.

If the individual has an ostomy or catheter for urinary control **OR** has an ostomy for bowel control, **but not both**, diapers or briefs and incontinence pads and under pads may be authorized. The frequency of the authorization would be based on the rating on the assessment of the function that qualifies the individual for the product. Example: An individual is rated Incontinent (total) for bladder and has a urinary ostomy and is rated Frequently Incontinent for bowel. **The frequency of the authorization would be every other month or twice per quarter based on the Frequently Incontinent for bowel rating rather than monthly based on the Incontinent (total) of bladder rating.**

Note: If an applicant/ participant has an ostomy or indwelling catheter for urinary control and experiences leakage of urine from the ostomy bag or around the indwelling catheter, the applicant/

participant may be authorized to receive under pads or incontinence pads based on the frequency of the leakage. If an applicant/ participant receives in and out catheterizations and experiences break through incontinence between catheterizations, the applicant/ participant may receive under pads and/or incontinence pads based on the frequency of leakage. Diapers and briefs should not be authorized. Approval must be obtained from the Division of Community and Facility Services RN and documented prior to referral.

## **V. Incontinence Assessments:**

Incontinence Supplies applications will be transferred to Incontinence Supplies Specialists and will display under the “Unassigned Centralized Intake Work Flow” tab in Phoenix. The Incontinence Supply Specialist will complete the phone assessment documenting the frequency of incontinence. The Specialist will also determine the type of supplies needed including the size and will document this information in the bowel/bladder section of the assessment. If an assessment is documented from CPCA, Community Choices, MCC or IS that was completed within **30 days** of the IS application, the Specialist may use this information to complete the IS phone assessment.

Incontinence Supplies assessments are due within 14 days of being assigned to the specialists. This due date will display under the Activity Due/Due Date column on the specialist dashboard under the Participants tab.

### **Steps to Complete an IS Assessment:**

- a. Access Phoenix and enter your Phoenix user ID
- b. Go to Unassigned Centralized Intake Workflows Search for cases in your assigned county Under Workflow Status column, click Awaiting Assignment to Area Nurse Select your name from the Assign to Area Nurse drop down, then select the Assign to Area Nurse button. The case will appear on your dashboard under Participants tab Select the participants’ case and check their dashboard to see if the message ‘Contact primary contact instead of participant!’ appears under Snapshot tab. If so:
- c. Select Participant Information then Primary Contact, write down the primary contact name and phone number
- d. View the participants general information and physician under the Participant Information tab
- e. Call the primary contact or participant as identified above
- f. Date the assessment only after you have reached the primary contact or participant and they have agreed to the assessment
- g. Go to the Assessments tab from the carousel and select the Create Phone assessment button. Select the program Incontinence Supplies and today’s date for assessed on.

- h. Proceed with the Medical Section questions for diagnosis, medical condition, height and weight. Ask participant to identify their primary care physician and to verify their address. Select Save and Continue.
- i. Proceed with ADLs section questions for toileting and bowel/bladder
- j. Ask participant what type of incontinence supplies and what size. Enter response in the comments section under bowel/bladder. Select Save and Continue.
- k. Proceed with Source of Information/Level of Care section questions. Sign and date the IS assessment in both spaces. Select Save and Continue.

Note: Attempt to contact the primary contact or participant two times for the Incontinence phone assessment. Narrate your call attempts. Send out a CLTC Notification letter after the second call attempt with a 10 day deadline for the primary contact or participant to return the call. If no contact after the 10 day deadline, terminate the case.

After the Incontinence Supplies assessment is completed, go to the Applications tab.

- If the participant is in another program then give the case back to the case manager after the Incontinence Supplies assessment is completed. You can find out who the case manager is by going to the Applications tab and look for the status participating next to the program.
- If the participant only has an Incontinence Supplies application and is not participating in another program, then assign the case to the designated Division of Community and Facility Services worker who authorizes Incontinence Supplies.

#### **How to assign a case after the Incontinence Supplies assessment is complete:**

- a) Go to the Applications tab
- b) Select Incontinence Supplies
- c) Once the Application for Incontinence Supplies populates, select Edit
- d) Under the Assigned Worker drop down select the name of person you want to assign case to
- e) Select Update

#### **VI. Pediatric Incontinence:**

Children ages 4 to 20 may exceed the state plan frequency limits for Incontinence Supplies per Early Periodic Screening Diagnosis and Treatment (EPSDT) policy if medically necessary. For children requiring more than the State plan frequency limits of total incontinence a re-assessment should be completed if the request is made after an initial assessment has already

been completed. The amount of incontinence supplies should be annotated in the comments section under bowel/bladder of the IS assessment. Include in the comments the justification for the additional incontinence supplies. Send approval of additional product to the Division of Community and Facility Services worker who authorizes Incontinence Supplies.

The following are some conditions that can be contributing factors to incontinence in children:

Attention Deficit Disorder/ Attention Deficit Hyperactivity Disorder

Urinary Tract Infection

Diabetes

Developmental Delays

Kidney Disorder

Neurological Disease

Cerebral Palsy

Seizure Disorder

Episodes of Focal Weakness

Constipation

Obstructive Sleep Apnea

Structure problems of the kidneys

## **VII. Choice of Service Provider:**

The participant and/or primary contact must make a choice of provider for incontinence supplies. The initial choice of incontinence supplies provider is documented on the participant's Provider Choice form. The Incontinence Supplies Specialists will:

- Print the Provider Choice form and mail to the applicant/primary contact.
- Print the Participant Cover Letter and mail to the applicant/primary contact.
- Upload the returned Provider Choice forms into Phoenix

The participant and/or primary contact may make the choice in writing on the Provider Choice form. It is important that the participant and/or primary contact makes this decision independently, and the assigned worker shall not influence the participant's choice of providers.

When the participant and/or primary contact request a change in providers, the assigned worker will inform the participant and/or primary contact of all available providers of the service. The assigned worker must narrate this information exchange as well as the choice of provider.

If DHHS and/or its designee does not receive the provider choice form within **14 days** of the date the provider choice form was printed and mailed to the participant, the incontinence application will be terminated. The revised provider choice form cover letter informs the participant of the 14 day timeframe. For DHHS case workers: The spreadsheet Provider Choice Forms Received may be used to track the timeframes; the spreadsheet is located on Sharepoint, under Community and Facility Services and in the Incontinence Supplies folder.

Once the provider choice has been obtained, a referral will be created and sent to the provider of choice.

#### **VIII. Physician Certification of Incontinence:**

The Physician Certification of Incontinence form serves as the Physician's Order for incontinence supplies. The Physician Certification of Incontinence form must be obtained by the provider of choice prior to any authorization of the supplies. The form must be signed by a Physician. The form may **not** be signed by a Nurse Practitioner, Registered Nurse or a Physician Assistant. The Physician's signature is the **only** allowable signature.

The incontinence supplies provider does not need to send a copy of the Physician Certification of Incontinence Form to DHHS and/or its designee; however, the provider must notify the assigned worker when the completed/ approved form has been returned from the Physician. Providers are able to go into the Phoenix provider portal and indicate that they have received the doctor's orders and tell how long the order covers. This will then trigger a notification to the assigned worker so that the worker can then create and send an authorization. The Incontinence Supply case may remain open pending supplies while the IS provider is awaiting the signed Physician Certification of Incontinence Form back. If the IS provider cannot obtain a Physician Certification of Incontinence Form back then the case must be terminated. A CLTC Notification form with an Appeals Notice must be mailed to the participant and/or primary contact to inform of the termination. The provider will get a termination notification by email in the Phoenix provider portal.

Normal process is the IS provider obtains the MD form. An exception if the IS provider is having difficulty obtaining the form then the beneficiary or PC can take a copy of the form to their doctor's office. As an additional step the case worker and/or vendor may contact the doctor's office to coordinate the MD form to the IS provider.

#### **IX. Review of Medicaid Eligibility Category and RSP:**

##### **Medicaid Eligibility Categories**

Prior to enrollment, the intake worker will review the applicant's Medicaid Eligibility category information. The applicant is determined to be financially eligible if the applicant is a current Medicaid recipient with any Medicaid category **EXCEPT** Categories 14, 48, 50 or 52 or 55. The applicant with Category 10 and 33 should be reviewed carefully to ensure the applicant is not residing in a skilled nursing facility and resides in the community, notate the date returned in Phoenix. Medicaid recipients who reside in a Community Residential Care Facility may receive Incontinence Supplies.

If the applicant has applied for Medicaid MAO Category 15, the applicant is not eligible for authorization of the incontinence supplies until the applicant has enrolled in a waiver. In this case, the participant may receive incontinence supplies during the 30 day wait period.

NOTE: When the applicant is approved for Medicaid Eligibility Category 15 MAO and is enrolled in the waiver, the referral should include a comment that the applicant is an approved Category 15 MAO participant who is in the 30 day wait period.

## RSP Codes

Prior to enrollment, the intake worker will also review the applicant's Medicaid Eligibility RSP information. When it is determined that an applicant is enrolled in a waiver or other special program, the Program Manager for that waiver or special program should be notified. Example: An applicant is referred for incontinence supplies and is noted to be enrolled in HSCE. The Program Manager for HSCE should be notified for direction.

## RSP Codes that DO OVERLAP with Incontinence Supplies:

| RSP  | Program      | Responsible for Authorizing |
|------|--------------|-----------------------------|
| AUTW | DDSN         | DDSN PPD Case Manager       |
| AUTS | DHHS         | DHHS IS Caseworker/Vendor   |
| TBR5 | DHHS         | DHHS IS Caseworker / Vendor |
| CHPC | CLTC         | CLTC Nurse Consultant       |
| CLTC | CLTC         | CLTC Case Manager           |
| CSWE | DDSN         | DDSN Case Manager           |
| CSWN | DDSN         | DDSN Case Manager           |
| DMRE | DDSN         | DDSN Case Manager           |
| DMRN | DDSN         | DDSN Case Manager           |
| HSCE | DDSN         | DDSN Case Manager           |
| HSCN | DDSN         | DDSN Case Manager           |
| ISED | DSS          | DSS Case Manager            |
| VENT | CLTC         | CLTC Nurse Consultant       |
| WMCC | MCC          | MCC Care Coordinator        |
| MCCM | Managed Care | Managed Care Plan           |
| MCHM | Managed Care | Managed Care Plan           |
| MCPR | PRIME        | Prime Case Worker           |
| MCSC | PACE         | PACE Program                |



**RSP Codes that DO NOT OVERLAP with Incontinence Supplies:**

| <b>RSP</b> | <b>Program</b> | <b>Coordinator</b> |
|------------|----------------|--------------------|
| MCHS       | Hospice        | Hospice Worker     |

Note: In addition to Hospice, participants in a nursing home or admitted to a hospital will not receive Incontinence supplies. The IS authorizations and application are to be terminated when the participant enters the nursing home. The worker assigned to the IS application will need to be notified that the participant has entered a nursing home so the terminations may be completed and notifications sent. Participants who are enrolled in Managed Care Plans (MCCM and MCHM) should be instructed to contact the Plan's Member Services regarding the need for incontinence supplies.

| <b>Managed Care Organization and Prime Member Service Phone Numbers</b> |                          |
|-------------------------------------------------------------------------|--------------------------|
| <b>MCO Name</b>                                                         | <b>Member Services #</b> |
| <b>Absolute Total Care</b>                                              | 866-433-6041             |
| <b>Advicare of South Carolina</b>                                       | 888-781-4371             |
| <b>First Choice by Select Health</b>                                    | 888-276-2020             |
| <b>Molina Healthcare of South Carolina</b>                              | 855-882-3901             |

**X. Enrollment in Incontinence Supplies Program:**

The applicant will be enrolled in the Incontinence Supplies Program when:

1. Financial eligibility has been determined; and
2. The Provider Choice has been obtained; and
3. Notification from the provider that the completed/approved Physician Certification of Incontinence form has been received.

The assigned worker will:

- Go to the Incontinence Supplies application in Phoenix
- Select Status Change

- Select Enroll and enter the date of enrollment
- The IS application status will show as Participating
- At this time there is not an RSP to key to indicate IS in MMIS
- For non-waiver participants, complete the CLTC Notification form indicating enrollment in Incontinence Supplies Program and mail to the Participant and/or Primary Contact. A notification form is not required for waiver, CPCA, or CPDN participants.

#### **XI. Authorization of Incontinence Supplies:**

Since Incontinence Supplies Program is not a waiver service, it is most important to remember the eligible Participant should not wait for subsequent application actions to be completed before the incontinence supplies are authorized. The Incontinence Supplies Program is independent of all other applications.

Example: An individual has an application for Incontinence Supplies and for Community Choices. The applicant has Medicaid Category 32, the assigned worker has been notified by the provider a signed Physician Certification of Incontinence form has been received. A signed Provider Choice form has also been obtained. The Eligibility Workflow has not been approved by Medicaid Eligibility and the Community Choices application is in progress. The assigned worker should proceed with authorizing the incontinence supplies while the case actions for Community Choices continue.

The assigned worker will review the assessment information that was completed by the Incontinence Supplies Specialist to determine the type, size and frequency for the incontinence supplies. This information may be found in the phone assessment. If clarification is needed to determine the type, size or frequency of supplies, the assigned worker will need to contact the participant/primary contact by telephone to clarify.

#### **Diapers, Briefs, Incontinence Pads, Under Pads**

The frequency of the delivery will be determined by the assessment rating for bowel and/or bladder. If at any time the assessment rating for bowel and/or bladder changes, the change should be annotated in the comment section of the assessment.

Authorization of diapers, briefs, incontinence pads and under pads must be based on frequency of incontinence as indicated on the assessment as follows:

- Occasionally incontinent allows up to one case per quarter
- Frequently incontinent allows up to one case every other month or up to two cases per quarter
- Totally incontinent allows up to one case per month
- Usually incontinent will not receive any supplies

The participant may receive 1 case of incontinence pads and 1 case of diapers or briefs at the same time for frequencies of frequent or total only, not for occasionally.

Bariatric Briefs have no billing code. However, the case manager may authorize Bariatric Diapers (using the Bariatric Diaper code) and add in the Comment section that Bariatric Briefs are needed, specifying 96 per case

#### **NO QUANTITY EXCEPTIONS MAY BE MADE FOR ADULTS AGE 21 AND OLDER**

##### **Gloves**

Gloves are not covered as an incontinence supply. Participants enrolled in the CS waiver, HASCI waiver and ID/RD waiver received extended state plan services for incontinence supplies. Gloves are covered under the DME program for participants in the above DDSN waivers.

##### **Wipes**

The participant must be incontinent to receive wipes. The authorization of wipes is NOT based on frequency of incontinence. The authorization of wipes is based on need and may be authorized up to one box monthly. **Wipes may only be authorized if the participant has an open authorization for diapers, briefs, incontinence pads or under pads.**

The delivery of the supplies will continue as long as the participant continues to meet the following 3 requirements:

1. Medically eligible for the incontinence supplies (at least occasionally incontinent of bowel and/or bladder
2. Financially eligible
3. Physician Certification of Incontinence completed and on file with the provider every 365 days or less depending on the selection chosen by the physician

If any one of these 3 requirements is not met, the service must be terminated. A CLTC Notification form must be mailed to the participant and/or primary contact with an Appeals Notice.

#### **XII. IS Participant Not Enrolled in a Waiver or Children's Personal Care Aide, or Children's Private Duty Nursing Program:**

When the IS Participant is determined to not be enrolled in CLTC Community Choices, HIV/AIDS, VENT, Children's Personal Care Aide or Children's Private Duty Nursing Program, but chooses to continue to participate in the Incontinence Supplies Program, the case will be transferred to an Incontinence Supplies Specialist.

If Incontinence Supplies Program continues to be appropriate, but not a waiver or Children's Personal Care Aide, the assigned worker will:

1. E-mail Specialists and Supervisor providing the CLTC Participant Number and Participant Name notifying of the program termination and need to continue supplies.
2. Do not terminate the Incontinence Supplies application or authorizations
3. The Specialist will assign the application to the appropriate person.

**XIII. IS Participant Enrolled in a Waiver:**

When the IS Applicant/Participant is determined to be enrolled in a waiver or Children's Personal Care Aide Program, the IS application will be assigned to the same person to whom the waiver application or Children's Personal Care Aide Program is assigned. The assigned worker will complete any and all of the steps required including enrollment in the IS Program and the authorization of supplies as needed.

If at any time the waiver enrollment or Children's Personal Care Aide Program is terminated and the Incontinence Supplies Program should continue, it is the assigned worker's responsibility to notify the Incontinence Supplies Specialist and follow the steps outlined in IS Participant Not Enrolled in a Waiver or Children's Personal Care Aide Program-.

The Specialist will assign the application to the appropriate person.

**XIV. Existing Waiver or Children's Personal Care Aide or Nursing Participant Who is NOT Enrolled in IS but Needs Incontinence Supplies:**

When an existing Waiver or Children's Personal Care Aide Program Participant who is NOT enrolled in Incontinence Supplies Program becomes incontinent and now needs incontinence supplies, the assigned Case Manager or Nurse Consultant will make an electronic referral to [https://phoenix.scdhhs.gov/cltc\\_referrals/new](https://phoenix.scdhhs.gov/cltc_referrals/new) for Incontinence Supplies Program.

Centralized Intake will create an Incontinent Supplies Program application and will complete the processing of this application as described in the Centralized Intake section. The necessary forms will be printed and mailed by the designated worker as described in the Application Process section. The application will then be assigned to the same worker that is assigned to the waiver application or Children's Personal Care Aide or Nursing Program application. The assigned worker will utilize the process as outlined in the Authorization of Incontinence Supplies section.

#### **XV. IS and Children's Personal Care Aide or Nursing Program:**

If Centralized Intake receives an application for the Children's Personal Care Aide Program and the Incontinence Supplies Program, the Children's Personal Care Aide Or Nursing application will be assigned to the Nurse Consultant. The application for Incontinence Supplies will be assigned to an Incontinence Specialist.

- If the MD indicates the child is appropriate for Incontinence Supplies but not for CPCA, The CPCA application will be terminated and the IS application remains open.
- If the returned forms indicate appropriate for CPCA but not Incontinence Supplies, continue with the CPCA application and terminate the IS application. Notify the participant and/or primary contact via CLTC Notification with the Appeals Notice.
- **At any time during this process if all requirements for the authorization of incontinence supplies are met, the supplies should be authorized. Authorization of supplies should not be delayed while processing CPCA.**

Authorizations for children ages 4 to 20 are based on the frequency of incontinence. For children requiring more than the set limits, the assigned worker must request an exception from the Division of Community and Facility Services worker. The request should include the Participant's Name, CLTC Number, frequency of incontinence and approximately how many diaper, briefs, incontinence pads, med pads or wipes are used daily. The increased supplies may only be authorized if the request for exception is approved.

#### **XVI. Termination:**

An applicant/participant should be terminated from Incontinence Supplies for any of the following reasons:

- Declined participation in program
- Died
- Entered administrative days
- Entered nursing home
- Entered nursing home with private payer
- Exceeded full calendar month\*
- Financially ineligible for Medicaid\*\*
- Inappropriate after intake
- Incarceration
- Intake criteria not met
- Medically ineligible for the program
- Moved outside of South Carolina
- Unable to locate the individual

**\*Full Calendar Month**

When a participant is closed for Full Calendar Month, the assigned Case Manager or Nurse Consultant will terminate the IS application when terminating the waiver application.

If/when the case is referred for re-enrollment for the waiver, the Centralized Intake Team will create the IS application if appropriate. The incontinence supplies should not be authorized for a participant who remains in a hospital or skilled nursing facility. The participant must be at home or in a community setting, such as a CRCF to receive the supplies.

**\*\*Financially Ineligible**

When a participant is closed for Financial Ineligibility, the assigned Case Manager or Nurse Consultant must determine if the financial ineligibility is for waiver eligibility only. If the participant continues to have Medicaid eligibility but not waiver eligibility, the waiver application will be terminated but the IS application is assigned to an IS Specialist.

A participant's Incontinence Supplies application will be transferred to the assigned worker/program for any of the following reasons:

- Entered Residential Care Facility
- Entered MCC Program
- Entered CPDN Program

## **XVII. Incontinence Supplies Administrative Case Management Policy - Vendors**

### **A. Vendor Solicitation Requirements:**

**Incontinence Supplies (IS):** Coverage of IS is under the mandatory Medicaid State Plan Home Health benefit. IS are part of the medical supplies, equipment and appliances component of Home Health. They are available to all Medicaid participants age four and older (up to any age) who meet the applicable medical necessity criteria, which include incontinence of bowel and/or bladder. A physician's order is required for participants to receive these supplies.

For both services, referrals are made to SCDHHS staff members. The referral is processed to determine if the participant meets the criteria for receiving the service(s). This includes a telephone assessment to determine whether the appropriate medical necessity criteria are met. For those participants who meet the criteria, a determination is made as to the level of services required.

For IS, the assessment determines the amount of supplies based upon the following criteria:

For adults age 21 or older, the frequency of incontinence and amount of supplies authorized are limited as follows:

- Occasionally incontinent allows up to one case per quarter. For bladder – occasional incontinence would be indicated by occurrences two or more times a week but not daily. For bowel – occasional incontinence would be indicated by occurrences once a week.
- Frequent incontinence allows up to two cases every quarter. For bladder or bowel – frequent incontinence would be indicated by daily incontinence, but some control or if the participant is being toileted (extensive assistance) on a regular basis, i.e. every two hours.
- Total incontinence allows one case per month. Total incontinence is indicated by no control (or an indwelling catheter or ostomy that controls the participant's bladder or bowel).

For children age 4 to 20, authorization of IS is based on frequency of incontinence and may exceed the Medicaid State Plan limits set for adults if medically necessary. Any exception for children must be requested and approved by SCDHHS prior to providing IS.

Contractor responsibilities begin once the participant is deemed eligible to receive the service and the frequency and amount of the authorization has been determined. In all cases, the Contractor will use SCDHHS' Phoenix system to provide and document activities.

- a. **Incontinence Supplies:** The Contractor will be assigned the case after the assessment and service plan have been created in Phoenix by the State Office. The Contractor will be notified in Phoenix of the amount of incontinence supplies to be authorized.

IS cases that are already being case managed will not be part of the scope of this solicitation and will not be assigned to the Contractor in Phoenix. The cases which are already being managed include IS cases that are part of Home and Community Based waivers such as Intellectual Disabilities/Related Disabilities waiver, Community Supports waiver, Head and Spinal Cord Injury waiver, Medically Complex Children's waiver, Community Choices waiver, HIV/AIDS waiver, Ventilator Dependent waiver or Managed Care cases. Contractor responsibilities include but not limited to the following:

- i. The Contractor will print and mail the provider choice form to the participant. If it is not returned within 14 days, the Contractor will attempt to contact the participant by mail and receive a choice of providers once. If this is unsuccessful, the case must be terminated using the Phoenix Notification form and a copy of the form is sent to the participant.
- ii. Upon receipt of the provider choice form, the Contractor will scan the form into Phoenix and manually enter the five selected providers from the provider choice form in the participants Phoenix record.
- iii. The Contractor will monitor Phoenix for acceptance of the referral by a provider.
- iv. The Contractor will use Phoenix to verify that the IS provider has obtained a signed and completed Physician Certification of Incontinence Form. The IS provider will be responsible for notifying the Contractor in Phoenix that the Physician Certification of Incontinence Form has been received. If the IS provider does not have a signed copy of the Physician Certification of Incontinence Form, then the Contractor cannot authorize the IS.
- v. Once the Contractor has received notification from the IS provider, the Contractor will authorize the IS in Phoenix as identified on the initial assessment.
- vi. The Contractor will be responsible for at least one phone contact every quarter with the participant regarding their satisfaction with the IS and IS provider. This quarterly contact must be documented in the narrative section of Phoenix and address at a minimum with whom the Contactor spoke (the participant or primary contact), the participant's condition, appropriateness of the IS and delivery of the IS.
- vii. The Contractor will be responsible for ensuring that the provider of IS has a current physician's order to cover the supplies both initially and for subsequent recertification periods.
- viii. The Contactor will be responsible for coordinating any changes in IS or IS provider if requested by the participant. A reassessment must be documented in Phoenix by the Contractor if a change in IS is requested.



- ix. Any increase in IS must be prior approved by SCDHHS.
- x. Any involuntary discontinuation of services must be approved in advance by SCDHHS.

## **B. How to Create a Referral for Incontinence Supplies**

1. Go to [https://phoenix.scdhhs.gov/cltc\\_referrals/new](https://phoenix.scdhhs.gov/cltc_referrals/new)
2. Click on green button, Create a New Referral
3. The New Referral page will populate
4. Complete all information on screen (ex. Applicant Information box)
5. Keep a record of the confirmation number given and the beneficiary name after you press Submit

## **c. Instructions for Entering IS providers after PCF received:**

1. Received completed, signed, and dated provider choice list
2. Check for eligibility and RSP codes (If MCHM showing Danita or Murray can check MMIS if you don't have access)
3. Go to Assessments (look for Incontinence Supplies Assessment, then check the bowel/bladder section) – to get type IS/frequency. If type IS/quantity not listed in assessment, check narrative). If not listed in narrative a phone call will have to be made to participant/PC to get type IS/frequency
4. Write down participant's name and type IS needed along with frequency
5. Go to Waiver Supports
6. Click on New Referral
7. Select Service (i.e. adult large briefs)
8. Click Next
9. Select first provider by clicking on green + (plus symbol) – Do this for all five providers
10. Click Next
11. Select Frequency, and any applicable notes (such as height weight, other IS needed, allergies, etc.)
12. Add Start Date (which should be two weeks from current date)
13. Click on Create Referral
14. Narrate

## **D. Instructions for Creating IS Authorizations**

### **Initial Steps**

1. Get Acknowledgement of Receipt of Physician Certification of Incontinence
2. Check for eligibility and RSP codes (If MCHM showing Danita or Murray can check MMIS if you don't have access)
3. Check application status (if initial – enroll in IS- make status participating)
4. \*If you made referral, Go to Waiver Services
5. Create authorizations (add to Notes section the weight and height of beneficiary) for each IS product needed
6. Create notification letter to PC/participant
7. Narrate (include the date, duration and IS provider from the Forms tab in Phoenix)

### **After IS provider responds:**

8. Check your notifications for “Physician Certification of Incontinence Form received for (CLTC 0123) Daffy Duck
9. Go to Forms and look under Acknowledgement column and notate date of signature and duration of certification
10. Go into Waiver Services under Active Referrals, click on view next to the active referral
11. Click on Approve Provider
12. Check the notes for other IS products (to create additional authorizations by clicking on New Authorization tab)
13. Enter the weight and height in Notes section
14. Enter the start date as the current date
15. Click on Create Authorization button
16. Click on Send to Provider button

\*If someone else made referral, go to step 3 under Instructions for Creating IS referrals and then follow steps 4 thru 8 under Instructions for Creating IS Authorizations

## **E. Instructions for Creating a Notification Letter in Phoenix:**

1. Go to Forms
2. Click on create a new form
3. Go to CLTC Notification Form
4. Select the address that has (mailing address, current next to it)
5. Refer to handout “Sample wording for Notification Letters” for language to add to Comments box
6. Sign the form

7. Click on Create Form

## **F. Instructions for Completing IS Checklist**

1. Go to Narrative
2. Click on New Narrative then Incontinence Supplies Quarterly Contact
3. Date of call
4. Prior to answering question 1, go to Claims and review the past 90 days of claims entered for each IS product.
5. Then enter the 90 day time frame under start and end dates, if no claims are present then ask the beneficiary if they have receive their supplies
6. Proceed with asking the beneficiary or PC the questions and selecting yes or no
7. Depending on the answer, another question will populate asking if changes have been documented in the narrative, use the box at the end of the checklist to narrate.
8. If the answer is no to the question, Are the amount of supplies being authorized sufficient, then follow the procedures for requesting an increase in IS by emailing Murray Goode (pediatrics) or Danita Goodman (adults up to Total).
9. If the answer is no to the question, Is the participant satisfied with the provider, then ask verbally if they want to choose another IS provider from their existing PCF and then proceed with coordinating the transfer of the MD form and authorizing the IS for the new IS provider and terminate for the old IS provider.
10. After the checklist is completed, on your dashboard the date will adjust for the next quarter (90 days from completed checklist) under Next QV Due

## **G. IS Policy for aging out**

If the case is not transferring to another program: Vendor will decrease the authorization down to the state plan max at the frequency already in place. If a change in frequency is needed then another IS assessment will need to be documented.

If the case is transferring to a CLTC waiver, OSS, etc. (with a new caseworker): Vendor will decrease the authorization down to the state plan max before transferring the case to the new caseworker.

If the case is transferring to a DDSN waiver, Managed Care, etc (program that does not use Phoenix): Vendor does not have to worry about changing the authorization because the case will be terminated.

## **H. IS Hold Policy**

During the quarterly contact call, if the participant or PC requests to stop IS that is considered voluntary termination, follow normal procedures for termination (no longer have to notify DHHS by email, proceed with narrating in Phoenix and send termination letter)

First request: If participant or PC requests a temporary hold on IS, Vendor will have to place hold on the authorization [by contacting IS provider via phone call or Phoenix conversation] and narrate

Second request: If at the next quarterly contact (now 6 months total) if the participant or PC requests to continue with the temporary hold at this point the Vendor will advise that the supplies cannot be held anymore. The Vendor can activate the authorization at the normal amount previously in place, decrease the supplies or terminate the case. If the vendor decreases and later the participant or PC wants to increase (back up to the original limits on the assessment) that is fine for them to do. Any increases above the limits on the assessment will need to be processed by Murray Goode (pediatrics) or Danita Goodman (adults up to Total).

## **I. Incontinence Supplies Frequency and Definitions**

When creating IS referrals and authorizations, please be mindful of the frequency (located under the Bowel/Bladder section of IS assessment).

- If rating for bowel and/or bladder is totally incontinent, participant's IS should be authorized on a monthly basis.
- If rating for bowel and/or bladder is frequently incontinent, participant's IS should be authorized one case diapers or briefs twice per quarter, chux quarterly, and three packs quarterly.
- If rating for bowel and/or bladder is occasionally incontinent, participant's IS should all be authorized quarterly.

\*Also please remember to create subsequent authorizations for med pads (chux) and wipes if they are listed as part of the notes on referral screen. Example- Will also need chux and wipes.

## **FREQUENTLY ASKED QUESTIONS (FAQs)**

- 1. Do we send a termination letter if participant is transferring to a DDSN waiver?**
  - a. Send a notification letter that their case is terminating due to DDSN enrollment and to contact their DDSN CM. (see enrollment language handout)
- 2. If participant is moving so no longer wants to participate?**
  - a. For documentation purposes send a notification letter at the address we have on file in Phoenix.
- 3. How do I ensure that I am paid for each IS case that I'm administratively monitoring as a vendor?**
  - a. Phoenix will auto-create authorizations for "Incontinence Supply Monitoring" for each case assigned to the vendors dashboard. You can view this under the Waiver Supports tab.
  - b. Whenever DHHS manually transfers IS applications to vendors the DHHS worker will create IS monitoring authorizations as well
- 4. How many wipes can a pediatric beneficiary receive?**
  - a. The maximum wipes allowed is 4 per month.
- 5. How can I communicate with DHHS Incontinence staff?**
  - a. You can send an email to DHHS Incontinence staff to request exceptions to policy and technical assistance. Please do not send a message through the Phoenix inbox. That method is OK for communicating with IS providers. Include the CLTC number in the email communication.
- 6. Are gloves covered under Incontinence Supplies?**
  - a. Gloves are not covered under Incontinence Supplies. It is considered a DME product. Gloves are covered under certain DDSN waivers.
- 7. Which Incontinence Supplies are covered through the State Plan Home Health service?**
  - a. The Medicaid State Plan Home Health benefit covers the following incontinence supplies as the maximum for an adult monthly:
    - One case of diapers or briefs (one case = 96 diapers or 80 briefs)
    - One case of incontinence pads/liners (chux) (one case = 130 pads)
    - One case of underpads (Poise pads)
    - One box of wipes
- 8. How can I verify the RSP information for Managed Care or one of the DDSN waivers and there's not an end date in Phoenix?**

- a. RSP information can be verified in MMIS. Please contact a DHHS Incontinence staff person by email.

**9. Are vendors responsible for the recertification of the Physician Certification Form?**

- a. Vendors are not responsible for policing the recertifications, this is the IS providers responsibility. The vendor's responsibilities are to make sure a Physician Certification Form is there before the initial authorization and to terminate the authorization if directed to by the IS provider.

**10. What do I do if the PCF is not received back within 14 days?**

- Call beneficiary and/or PC, if phone disconnected send a notification letter with a 10 day deadline asking for a return call and an updated phone number to discuss PCF. If no response then terminate
- Call beneficiary and/or PC, leave message if no return call and call back a week later for the second and final call attempt. If no response then send termination letter (please note do not send a 10 day notification letter).

**11. How do I request an IS increase for adults and/or children?**

- a. For adult increases (up to Total), contact DHHS Incontinence staff by email with the CLTC# and the supplies requested. DHHS Incontinence staff will attempt two phone contacts with the beneficiary and/or PC to perform the reassessment. If no contact then the request will be denied and an email will be sent to vendor. Please note, DHHS Incontinence staff will not send a notification letter to the beneficiary.

**12. Can a beneficiary get wipes only?**

- a. No, beneficiaries cannot receive wipes only. They have to be accompanied by an open authorization for diapers, briefs, incontinence pads or under pads.

**13. Does Incontinence Supplies require a Service Plan?**

- a. No, only if the beneficiary also has Children's Personal Care and then the service plan is only updated for changes.

**14. Do I need to contact DHHS for a change in size of IS?**

- a. No, for a change in size, vendors can go ahead and change. Contact DHHS for an increase (change in frequency) or additional IS items.

**15. Do I need to terminate IS before I transfer the case to another program's caseworker?**

- a. No, do not terminate IS applications or authorizations if a case needs to be transferred. Only terminate the IS monitoring authorization.

**16. Participant in the community is getting IS and then is admitted to a hospital and has to go to a nursing home. What happens to the IS?**

The IS authorizations and application are to be terminated. The worker assigned to the IS application will need to complete the terminations and send out notifications. Vendors must also terminate the IS monitoring authorization.

**17. Can I receive payment if I have attempted to contact the participant to perform the quarterly IS checklist but have been unsuccessful?**

- a. Yes, you can receive payment. Your billing report will show a status of FALSE for each quarterly checklist that was due and not performed. In order to receive payment, you will have to narrate three (3) times in Phoenix that you have attempted to contact the participant and what the outcome was i.e. left a voicemail. The three (3) contact attempts must occur throughout the month with three separate dates narrated. Narration of the three (3) contact attempts occurring in the last week of the month will not be accepted.